جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايمان محمود احمد قاسم
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** نصار
رقم البطاقة : 26706042601126 **رقم التليفون :** 01010231980 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 04-06-1967 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايمان محمود احمد قاسم
National ID: 26706042601126

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Pat. English Name: Eman Mahmoud Ahmed	Pat. Age 58 Y
Study Date 04/01/2026	Pat. ID: 2958748
Procedure Name: MRI Brain	Ref. Phy. Rotana Skan

MRI Study of the Brain With Contrast

Technique

- Axial ***T1, T2*** *and FLAIR WIs.*
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Findings:

- Right cerebellum intra axial rather well defined irregular shape SOL measures 3.4x4.3cm, it elicits slight high T2 and low T1 WI signal with heterogenous post-contrast enhancement.
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- Right frontal ischemic focus.

Yours truly

Dr. Amr Abd Al Qader, MD

Typed by: Aisha
Revised by: Dr.D

M.R.I
[[DOCTYPE:MRI]]

Name: ايمان محمود احمد قاسم **MR. :** 3466	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has bilateral breast cancer with metastases to the liver, bone, and newly identified brain lesions. Ten fractions of IMRT to the brain were completed between February 2 and February 14, 2026. The most recent multidisciplinary team discussion on January 15, 2026 recommended systemic therapy with XELODA and TYKERB. On March 11, duplex ultrasonography of the right lower limb demonstrated subacute thrombosis of the calf vein with associated edema; the patient was started on Eliquis 5 mg twice daily. The treatment plan at that time included cycle 3 of XELODA (5 days on, 14 days off) and cycle 2 of TYKERB (5 days on, 30 days off), pending laboratory results.

A bone scan performed in April 2026 showed a stationary course of previously known sternal lesions. A new CT of the chest, abdomen, and pelvis was ordered to assess response to XELODA and TYKERB, with laboratory studies pending.

On June 23, 2026, the patient was noted to be on cycle 5 of XELODA and TYKERB for metastatic breast cancer. Examination revealed redness of the right breast, raising concern for cellulitis versus local disease progression. A breast ultrasound was ordered, and antibiotics were prescribed. The next clinic visit was scheduled for July 14, 2026.

On September 1, 2026, the patient was evaluated for brain metastases with a scheduled MRI of the brain. The treatment plan included continuation of XELODA (dosed as 3 × 2 × 14) and TYKERB. The next follow-up appointment was set for September 22, 2026.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 15/1/2025 G

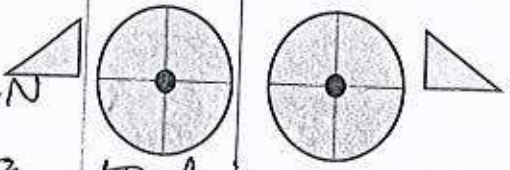
Name	ايمان محمود احمد	MRN	2474/2024
		ID	87b.7.E57-4C1
Age	58 yrs	Phone	01010231982
Menopausal status	Post.	Marital status	
NRS SCORE		Family history	+ ve Canc
BMI	Name	REASON OF MDT	Prog new BRAIN mets
CO morbidity	xDM xHTN		

Clinical Data

BX: Bilateral Br Ca

ER:	PR:	HER 2:	Ki67:
518	018	+3	

R PSI L



Sonomammography: Rt ← → left (1) (1)

Denovo met Bil. Breast Ca to liver + bone

Mets Workup: pt started weekly Taxol/ Carbo in 9/24 → 3rd week (+ Herceptin 6 cycles) ended in 4/6/2025, TNM

Kept on IV Herceptin ~~→~~ CT CAP 8/25

Recommendations: Shared RD of HFL + SD of rest of study

Lost MDT → to be kept on maintenance Herceptin + AI

pt refused 1b cycle Herceptin + AI, last on 31/12

C/O of vomiting, Dizziness + Headache → MRI BRAIN

MRI B → Rt cerebellar intraaxial SOL w/ mass effect 3.4 x 4.3 cm, pt on Methabigine, sx improved 4/1/26

MDT OUTCOME & PLAN Rx next plan of action ... to be kept on Herceptin + AI

1) Tykerb + Xeloda

2) possibility of Local rtx to brain

10/1/26

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form

Patient Name	: الاسم / ايمان محمود احمد قسم	Request No	: 13007944
Patient No.	: 26706042601126	Request Date	: 01/08/2024
Path No.	: SR-1065/24	Verification Date	: 19/08/2024
Sex	: Female	Financial Category	:
Age	: 57 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer and Left axillary lymph node tru-cut biopsies
Nature of Specimen: Slide Revision.

Gross Examination:
- Two paraffin blocks labeled by (20124) and three slides were received.

Microscopic Examination:
1- Right breast mass:
- Histological type: Invasive lobular carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- Ductal carcinoma in situ: Not present.
- Perineural invasion: Not present.
- Angioinvasion: Not present.
- Microcalcifications: Not present.

2- Left axillary lymph node:
Sections examined reveal lymphoid tissue cores infiltrated by malignant epithelial cells arranged in files and individual cells.
- Revised IPT:
- ER: Intermediate nuclear reaction in 15% of tumor cells (score 5/8).
- PR: No nuclear reaction of tumor cells (score 0/8).
- Her2: Intense circumferential membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:
1-Right breast mass, Slide Revision:
Invasive lobular carcinoma, Grade II.
2- Left axillary LN, Slide revision:
Metastatic lobular carcinoma.
IPT of both biopsies:

Verified by :
Dr.Rofayda Abdallah

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - ميدان القهوة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mini El Qahwa - Cairo, Egypt

Patient Name	: الأستاذه / ايمان محمود احمد قاسم	Request No.	: 13007944
Patient No.	: 26706042601126	Request Date	: 01/08/2024
Path No.	: SR-1065/24	Verification Date	: 19/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 57 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

- ER: Positive (score 5/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).

Signature
Dr. Salma Tarek.
Dr. Rufaida Abdullah.

Verified by :
Dr.Rofayda Abdallah

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name : Eman Mahmoud Ahmed Kasem	Date : 10-12-2025 12:52
Branch : Portable	Medical No. : GRI-95293
Ref.Physician Prof :	Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of managed bilateral cancer breast, compared to the last CT examination dated 6/8/2025 (hard copy with no DICOM series), the current examination revealed

I- CHEST:

- No significant time interval changes regarding bilateral breast subcutaneous edema and skin thickness.
- The previously noted right middle lobe consolidative area is no longer seen in today's study.
- Still noted right upper lung lobe anterior segment centrilobular emphysematous bulla measures about 13x6 mm.
- No significant time interval changes of left upper lobe small calcified nodule measuring about 3 mm.
- No significant time interval changes of few tiny calcified mediastinal lymph nodes.
- Bilateral basal lung lobes atelectatic bands,
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation.
- No significant time interval changes of bilobar ill defined hypodense hepatic focal lesions for further dynamic MRI
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستافروسي انترناشونال

Name : Eman Mahmoud Ahmed Kasem	**Date : 10-12-2025 12:52**
Branch : Portable	**Medical No. : GRI-95293**
Ref.Physician Prof :	**Ins.Company :**

- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Bulky uterus showing homogeneous myometrium with no focal lesions.
- Fat containing small umbilical hernia still noted.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton showed no worrisome lytic or sclerotic lesions,
  - Still noted sclerotic foci at D5 and D8 vertebral bodies as well as upper sternal body.
  - First degree anterolisthesis of L4 over L5 and of L5 over S1 vertebrae.
  - L5 right pars break.

Opinion:

- **Resolution of right middle lung lobe consolidative area**
- **Stationary course of the rest of the examination.**
- **For further dynamic MRI.**

Dr.Norhan Ramzy,MD

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Eman Mahmoud Ahmed Kasem	Date : 06/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 58 Year(s)
Medical ID: 2474/2024	Pat. ID: 95293

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to last CT dated 22/3/2025

CHEST FINDINGS:

- **Stationary** diffuse edema and thickening of both breasts.
- **Complete resolution** of the moderate degree right pleural effusion .
- **Regression of** right middle lobe consolidative patch area with an air bronchogram.
- **Stationary course** of left upper lobe small calcified nodule measuring about 3 mm.
- **Stationary course** of few tiny calcified mediastinal lymph nodes.
- Right upper lobe emphysematous bulla 13 x 6mm.
- No left side pleural or pericardial effusion.
- Patent trachea and major bronchi.
- Normal CT appearance of the heart and major thoracic vessels.

ABDOMEN AND PELVIS FINDINGS:

- The liver is average in size showing inhomogeneous parenchyma with **regressive course** of the multiple bilobar hepatic focal lesions, the largest one is seen at segment VI measuring about 8 x6 mm compared to about 1x0.9 cm in the prior study.
- Patent homogenously enhancing portal vein and is main branches.
- No intra-hepatic or extra-hepatic biliary dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No sizeable pathologically enlarged abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both adrenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus showing homogeneous myometrium with no focal lesions.
- Fat containing small umbilical hernia still noted.

1. ***The scanned skeleton:***

- **Stationary course** of sclerotic foci at D5 and D8 vertebral bodies as well as upper sternal body.
- First degree anterolisthesis of L4 over L5 and of L5 over S1 vertebrae.
- L5 right pars break.
- No vertebral structural collapse or extra-osseous soft tissue component.

CT Scan
[[DOCTYPE:CT_SCAN]]

BAHEYA
Radiology Department

قسم الأشعة التشخيصية

PATIENT NAME: EMAN MAHMOUD AHMED KASEM
GENDER: Female
DATE : 10-Feb-2025 02:01 PM

PATIENT ID: 20224009308
AGE: 57 y
ORDER: 3247981

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 8/9/2024, the current study revealed stationary course of the equivocal sternal lesion.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

Stationary course with no newly developed osseous lesions detected.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant **12-Feb-2025 03:45 PM**

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٤ شارع سنوية أول سابع الهرم بجوار داوى، الجيزة
مستشفى بهية الشيخ زايد
الحي الأول - الشيخ زايد أمام كلية الهندسة
Bone Scan
[[DOCTYPE:BONE_SCAN]]

Pat. English Name: Eman Mahmoud Ahmed	Pat. Age 58 Y
Study Date 04/01/2026	Pat. ID: 2958748
Procedure Name: MRI Brain	Ref. Phy. Rotana Skan

MRI Study of the Brain With Contrast

Technique

- Axial ***T1, T2*** *and FLAIR WIs.*
- *Sagittal* ***T1*** and coronal **T2** WIs.

Findings:

- Right cerebellum intra axial rather well defined irregular shape SOL measures 3.4x4.3cm, it elicits slight high T2 and low T1 WI signal with heterogenous post-contrast enhancement.
- Such lesion seen surrounding with moderate edema exerting together positive mass effect the form of compression of the 4th ventricle causing consequent supra tentorial moderate hydrocephalus.
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- Right cerebellum intra axial SOL with mass effect as described ,in view of patient history metastasis comes on top of the DD list for clinical correlation and follow up.
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Revised by: Dr/D

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